8.9 Hypothyroidism*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I feel cold, tired and slow." **Objective** • High TSH, low T4 • Weight gain • Dry skin	**ACTUAL** Excessive fatigue burden related to decreased metabolic rate as evidenced by tiredness and slowed activity.	• More energy within weeks of starting treatment.	• Give levothyroxine in the morning on an empty stomach. • Explain that treatment is lifelong, with TSH checks from time to time. • Plan rest.	• Goal reached as energy improves.
Subjective • "I have not opened my bowels for 4 days." **Objective** • Sluggish bowel sounds • Hard stools	**ACTUAL** Impaired intestinal elimination related to slowed gastrointestinal motility as evidenced by infrequent stools.	• Regular soft bowel actions within 3 days.	• Encourage fluid and fiber. • Encourage activity as tolerated. • Give the prescribed laxative.	• Met when bowel pattern is regular.
Subjective *No specific complaint* **Objective** • HR 52/min • Older adult	**RISK** Risk for decreased cardiac output as evidenced by risk factors: bradycardia and myxedema.	• HR and BP stay stable.	• Avoid sedatives. • Start thyroxine at a low dose in older or cardiac patients, as ordered. • Report chest pain.	• Achieved while she remains stable.

*References for this care plan: [32], [35], [37], [126]. Full entries are listed in the References section of the CPMN book.